



Comfort Room Use SOP (Mental Health)

Scope

Site	Department, Division or Operational Area	Applicable to
Royal Perth Bentley Group (RPBG)	All mental health (MH) inpatient areas	Nursing, Allied Health and Medical staff

Purpose

The purpose of this standard operational procedure (SOP) is to provide guidance for the safe use of patient comfort rooms/quiet areas within MH inpatient areas. Comfort rooms or low stimulus areas are to be made accessible for patients as part of management of emotional regulation and distress reduction.

Definitions

Comfort room/Separate area	A separate area or a purpose built room of the inpatient unit designed to help calm a patient when they feel stressed / distressed / aggressive / overwhelmed and / or to prevent exacerbation of a crisis and that may include: • Sensory modulation or quiet areas • Activity and outdoor areas • Decor that encourages relaxation, e.g. comfortable furnishings that minimise risk to self/others • Low light levels • Capacity to play appropriate soothing music • Resources, e.g. weighted blankets, stress balls and sensory items • Appropriate temperature to facilitate a warm and comfortable environment. The room/area is to support de-escalation strategies to give the patient an opportunity to choose to separate from others for a period of time. The comfort room will be available 24 hours per day, 7 days per week.
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Version: 3
Endorsed by: Co Director Mental Health 29/10/2024
Authorised by: Clinical Practice Committee 07/11/2024

Date Compiled: 06/2018
Executive Sponsor: Coordinator of Mental Health Nursing
Revision Due: 11/2027

Procedure details

A range of spaces designed for therapeutic requirements for patients **must** be provided in **authorised inpatient MH services** to align with the [Chief Psychiatrist's Standards for Authorisation of Hospitals under the Mental Health Act \(MHA\) 2014](#) and in **unauthorised MH inpatient services**, wherever possible.

Clinicians are to:

- Use a trauma informed approach to care for patients and take active steps to ensure their safety, particularly patients who have experienced trauma and are at risk of further trauma
- Seek patient collaboration and/or support a patient's request to access a comfort room, as soon as possible after admission and during treatment, where appropriate. Consider patient use of a comfort room:
 - in parallel to other de-escalation strategies
 - to facilitate therapeutic alliance
 - at their personal support persons (PSPs) request (with consent)
 - as an alternative to restraint and seclusion if this can be safely achieved
- Complete or review recent risk assessment (e.g. *Risk Assessment Management Plan [RAMP]*) in accordance with the **Mental Health Inpatient Triage to Discharge SOP** and/or **Progressive Risk Assessment [PRA] SOP** prior to using a comfort room and document agreed outcomes in the patient's progress notes and on the *Treatment, Support and Discharge Plan (TSDP)*
 - *Determine if some items may need to be secured inside cupboards for safety and/or access not granted*
 - If the patient is not suitable for comfort room use due to clinical reasons, other areas can be encouraged for de-escalation activities and providing a low stimulus environment. An explanation must be documented in the progress notes and clearly explained to the patient and/or PSP
- Consider exclusion criteria for the comfort room use for patients posing an infectious risk
- Identify and refer to the patient's care plan and patient-driven strategies to prevent or reduce distress or agitation or self-regulation of emotions. Also refer to the *What Matters Most (WMM) form* or *TSDP*
- Once the patient is assessed as suitable to use the comfort room, inform the Shift Coordinator and document the comfort room use in the patient's progress notes (prior to and after use) which may include:
 - Time in and out
 - Patients self report of benefits or self rated level of alertness/distress
 - Staff assessment of patient's level of alertness/distress
- Customise the comfort room with items that the patient has deemed to be therapeutic, where possible. Food and drink or personal items are not permitted in the room unless specified as an intervention in the *TSDP/WMM form* or as a medical directive, e.g. meal support

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Procedure details cont'd

- Ensure the comfort room is used by **one patient at a time** and depending on the demand for the room, may be time limited
- Certain equipment and techniques are best used by practitioners with training in their safe and skilled use, e.g. weighted blanket. Consider liaising with Occupational Therapy (OT) for assistance or other, suitably qualified staff for specialised clinical sensory interventions for patients
- Provide the patient with orientation and instruction on expected behaviour and outcomes of comfort room use
- Ensure **patients may voluntarily leave the comfort room at any time**
- Inform the patient of the need for regular observations during comfort room use and maintain discreet observations at a minimum of every 15 minutes to monitor safety and efficacy. Level of observation may be increased depending on risk
 - Closed circuit television (CCTV) may be used where available
- Record observations in the *Visual Observation Chart* and document the patient's comfort room use
- Escalate care to the Shift Coordinator, Senior Nurse and Medical Officer/ Consultant, if required
- Utilise ISoBAR clinical handovers, as necessary
- Ensure comfort rooms are locked while not in use, unless otherwise determined by the individual ward/ unit
- Ensure staff and patients adhere to area hand hygiene and infection control requirements
- After use, the clinician who facilitated comfort room access is to ensure:
 - the space and equipment/items are in orderly condition and clean and as per infection control requirements
 - in the event of an infection outbreak consult with Infection Prevention & Management staff about removal of certain equipment
- Nursing staff to complete comfort room inventory of equipment fortnightly and remove/ report faulty equipment to OT staff and/or escalate to Nurse Unit Managers (NUM)/Clinical Nurse Specialist (CNS) for repairs/ replacement of missing faulty items
- Practice within the Mental Health Act (MHA) 2014 and the [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- Complete a Datix Clinical Incident Management System (CIMS), e.g. clinical incident related to the use of comfort rooms/activities and/or Combined Hazard or Incident Report (CHoIR)/Notifiable Incident Reporting to the Office of the Chief Psychiatrist (OCP), if required.

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Compliance monitoring

Audit of monitoring forms with time used, impact on level of distress will be indicators of effective use of comfort rooms. NUMs/CNSs will monitor use of comfort rooms as part of the review of CIMs and episodes of restraint and/or seclusion.

Compliance will be monitored as part of the:

- Datix CIMS system process (e.g. clinical incidents relating to comfort room use)/ CHoIR /Notifiable Incident Reporting to the OCP, if required
- Customer service feedback.

Facilitator

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Related policies, practice standards, clinical guidelines

[RPBG Policy Hub](#)

- [Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP](#)
- [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Mental Health Restraint and Seclusion Policy](#)
- [Standard and Transmission-based Precautions SOP](#)
- [Mental Health Inpatient Triage to Discharge SOP](#)
- [Progressive Risk Assessment \[PRA\] SOP](#)

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Related policies, practice standards, clinical guidelines cont'd

EMHS Policy Hub

- [Care Coordination in Mental Health Services Policy](#)
- [Infection Prevention and Management Policy](#)

Department of Health Policy Hub

- [Clinical Incident Management Policy](#)
- [Statewide Standardised Clinical Documentation](#)
- [Safety Planning for Mental Health Patients Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Patients](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- [Chief Psychiatrist's Standards for Authorisation of Hospitals under the Mental Health Act \(MHA\) 2014](#)
- [Mental Health Act 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)

Related forms

- *Risk Assessment Management Plan [RAMP]*
- *Treatment, Support and Discharge Plan (TSDP)*
- *What Matters Most (WMM) form*
- *Visual Observation Chart*

Legislative requirements, the evidence and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy HUB.

ACSQHC NSQHS Standards 2nd Edition (2021)

Standard 1: Clinical Governance;
Standard 2: Partnering with Patients;
Standard 5: Comprehensive Care;
Standard 6: Communicating for Safety

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